

TITLE:

A phase I study of [REDACTED] in combination with mFOLFOX6 in subjects with CLDN18.2-expressing gastrointestinal adenocarcinoma

Sponsor Protocol Number: [REDACTED]

Investigational Product: [REDACTED]

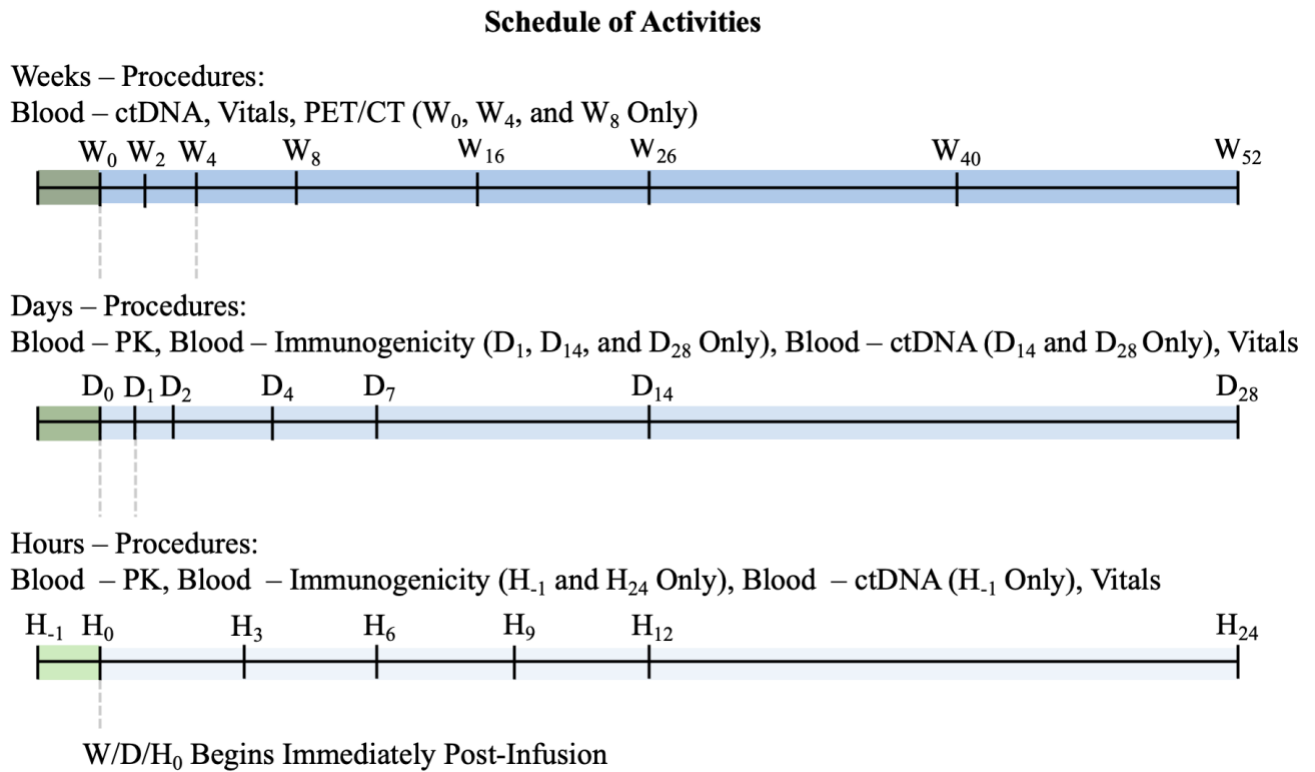
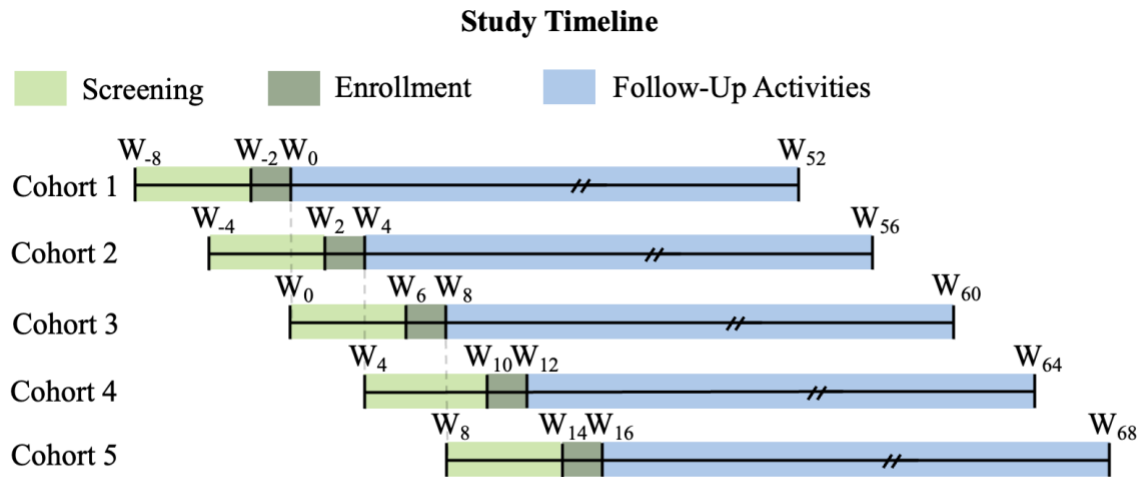
Sponsor: [REDACTED]

Trial Phase: I

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SCHEMA



SCHEDULE OF EVENTS

Activity	Screening	W ₀	W ₂	W ₄	W ₆	W ₈	W ₁₀	W ₁₂	W ₁₄	W ₁₆	W ₂₆	W ₄₀	W ₅₂
Informed Consent	X												
Demographics and Medical History	X												
Physical Exam	X									C	C	C	C
Height	X												
Weight	X	X	X	X		X				X	X	X	X
ECOG Performance Status [†]	X									C	C	C	C
Pregnancy Test [‡]	X												
Clinical Laboratory Tests	X												
Investigational Product Infusion		X											
		X	R	R	R	R	R	R	R	R	R	R	R
Vital Signs	X	X	X	X		X				X	X	X	X
Blood Sample (PK) ^a		X	X	X									
Blood Sample (Immunogenicity) ^b		X	X	X									
Blood Sample (ctDNA) ^c		X	X	X		X				X	X	X	X
PET/CT	X	X		X		X				C	C	C	C
AE/SAE Evaluation		X											

X: Activity performed as scheduled.

R: Activity recommended every 2 weeks or as clinically appropriate for standard of care.

C: Activity performed as clinically needed.

a: Samples collected prior to infusion (-1 hour), after infusion (0 hours), and at 3, 6, 9, 12, 24 hours and 2, 4, 7, 14, 28 days (W₀, W₂, and W₄).

b: Samples collected prior to infusion (-1 hour) and at 24 hours, 2 weeks, and 4 weeks (W₀, W₂, and W₄).

c: Samples collected for ctDNA prior to infusion (-1 hour) (W₀) and at 2, 4, 8, 16, 26, 40, and 52 weeks.

†: Performance evaluation status required at baseline.

‡: Pregnancy test for women of childbearing potential.

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1 PROTOCOL SYNOPSIS

1.1 Protocol Synopsis

The goal of this trial is to learn if a single intravenous (IV) injection of the investigational drug, [REDACTED] is safe and tolerable when administered in combination with standard-of-care mFOLFOX6 (modified folinic acid, fluorouracil, and oxaliplatin) chemotherapy in adult patients with CLDN18.2-expressing gastrointestinal (GI) adenocarcinoma.

The study will measure differences in safety and tolerability of a single dose of [REDACTED] at increasing doses when administered in combination with mFOLFOX6. The primary objectives of the study are to determine if a single injection of [REDACTED] with mFOLFOX6 safe and to measure the concentration of [REDACTED] in the blood of participants over time. The secondary and exploratory objectives of the study aim to evaluate the immunogenicity and pharmacokinetics (PK) of [REDACTED] as well as any antitumor benefits conferred.

Researchers will compare the investigational therapy ([REDACTED] in combination mFOLFOX6) to the standard-of-care (mFOLFOX6 with placebo).

1.2 Trial Schema

This Phase I adaptive trial is a randomized, single-blind, dose-escalation study. It is designed to be a rule-based assessment of dose-limiting toxicity (DLT) to identify the maximum tolerated dose of maximum feasible dose (MTD/MFD) of [REDACTED] administered in combination with mFOLFOX6 chemotherapy.

AE severity will be graded according to the metrics of the Common Terminology Criteria for Adverse Events (CTCAE) v6.0.¹ DLT will be defined as the occurrence of any adverse event(s) (AE) $\geq$ grade 3 considered treatment-related adverse events (TRAEs) possibility related to [REDACTED] that occur(s) any time from the initial dose of investigational treatment with [REDACTED] to the conclusion of the study.

Dose escalation will proceed within each cohort of 4 participants (randomized 3:1 [REDACTED] to placebo) according to the following schema:

Number of Participants Randomized to [REDACTED] with DLT	Escalation Decision
0 out of 3	Enroll and randomize 4 participants at the next dose level.
1 out of 3	4 additional participants will be enrolled. If 1 $\geq$ of the additional participants receiving [REDACTED] experiences a DLT ($2 \geq 6$), no additional participants will be enrolled, and this will be considered the MTD. If 0 of the additional participants experiences a DLT, enroll 4 participants at the next dose level.

2 $\geq$ out of 3	No additional participants will be enrolled. This will be considered the MTD.
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1.3 Schedule of Activities

The study will consist of screening, enrollment, and randomization followed by baseline assessment, investigational treatment, and follow-up assessments up to 12 months post-infusion.

Baseline assessments: Blood samples, PET/CT images, and assessments of physical vitals will be collected for all eligible participants who are enrolled and randomized into the study to establish baseline measurements post-infusion. Blood samples prior to infusion (-1 hour) will be used to establish pre-treatment complete blood count (CBC), antibody abundance, and circulating tumor DNA (ctDNA) levels. PET/CT scans will establish baseline cancer anatomy.

Follow-up assessments: Blood samples will be drawn prior to infusion (-1 hour), upon completion of infusion (0 hours) and at 3, 6, 9, 12, 24 hours and 2, 4, 7, 14, 28 days (in weeks 0, 1, 2, 4) post-infusion for PK measurements. Sera will be isolated from blood samples collected prior to infusion (-1 hour) and 24 hours, 14 days, and 28 days (in weeks 0, 1, 2, 4) post-infusion for immunogenicity measurements. Additional blood samples will be collected 2, 4, 8, 16, 26, 40, 52 weeks post-infusion to enable measurements of ctDNA levels. PET/CT scans will be completed post-infusion, 4 weeks post-infusion, and 8 weeks post-infusion. Physical vitals will be measured at each clinic visit.

2 INTRODUCTION

2.1 Purpose of the Trial

Gastrointestinal (GI) cancers account for over a quarter of all global cancer cases and is the fifth most common cancer worldwide.^{2,3} Over one-third of all cancer-related deaths are associated with GI cancers.³ The number of new cases of GI cancers has steadily increased and is expected to continue increasing as the average global lifespan rises, since the risk of GI cancers increases with age.^{4,5} The incidence of early-onset (EO) GI cancers also continues to rise globally.⁶

Surgery remains a first-in-line treatment for removing primary tumor masses and chemotherapy the common standard of care for patients with later-stage diagnoses and/or metastases. Yet the prognosis with these therapies alone or in combination often remains poor. The median overall survival (OS) of patients with metastatic gastric adenocarcinomas who receive surgery, chemotherapy, or a combination of surgery and chemotherapy has been measured to be 3.9, 7.0, and 14.2 months, respectively.⁷ Tumor recurrence and chemotherapy-related complications are often causes associated with treatment failure.³

CLDN18.2 has been recognized as an attractive target for GI cancer therapies.⁸⁻¹⁰ Altered expression of CLDN18.2 has been observed in many GI cancer samples and CLDN18.2 is often

exposed on the surface of malignant cells.⁹ Because CLDN18.2 is almost exclusively located in the tight-junction supramolecular complexes in normal tissues, therapies targeting CLDN18.2 are well-poised to have minimal off-target effects.^{8,9}

Many CLDN18.2 therapies have been and continue to be developed to treat GI cancers.³ Immunoglobulin G1 (IgG1) monoclonal antibodies (mAbs) targeting CLDN18.2 have been shown to improve clinical outcomes in cancer patients when used in combination with chemotherapy. Zolbetuximab improved progression-free survival (PFS) and overall survival (OS) in combination with mFOLFOX6 (modified folinic acid, fluorouracil, and oxaliplatin) in the SPOTLIGHT (NCT03504397) trial.¹¹ Similar improvements to PFS and OS were observed with capecitabine plus oxaliplatin (CAPOX) in the GLOW (NCT03653507) trial.¹² In a phase I/II trial (NCT04632108), ASKB589 in combination with capecitabine plus oxaliplatin lead to a disease control rate (DCR) of 100% (12 of 12 participants).¹³

The efficacy observed by CLDN18.2 monoclonal antibodies therapies has been attributed to activation of the antibody-dependent cellular toxicity (ADCC) and complement-dependent cytotoxicity (CDC) pathways mediated by the specificity of the antibody to tumor cells highly expressing CLDN18.2.¹⁴

██████████ is a humanized IgG1 monoclonal antibody that has been developed as an anti-CLDN18.2 therapy with the goal of reduced immunogenicity. This trial seeks to assess the safety profile of ██████████ in combination with chemotherapy when given to patients with high-grade CLDN18.2 expressing GI cancers. If successful, the efficacy of ██████████ in combination with chemotherapies may be evaluated in phase II trials.

2.2 Assessment of Risks and Benefits

2.2.1 Risk Summary and Mitigation Strategy

Trial Intervention – Monoclonal antibodies (mAbs) are non-endogenous proteins associated with immunogenic and hypersensitivity risks.¹⁵ ██████████ is a humanized mAb engineered to decrease the amount of non-endogenous protein introduced during infusion and therefore decrease its immunological potential. Hypersensitivity/adverse reactions associated with other chimeric or humanized mAbs used for cancer therapy frequently include cutaneous reactions such as rash, dermatitis, urticaria, pruritus, and alopecia.^{15,16} Nausea and vomiting were common treatment-related adverse events (TRAEs) observed in previous trials using mAbs to treat gastrointestinal adenocarcinoma.¹³

Trial Procedures – Blood sampling is a noninvasive, low-risk procedure. To minimize local complications and infection related to sampling blood, phlebotomy will only be completed by trained healthcare professionals or clinical study staff who have completed training and have documented competency. PET/CT scans will use as low as reasonably achievable (ALARA) principles of radiation safety and to minimize participant exposure to radiation. CT contrast will not be used for participants at risk for nephrotoxicity in the professional opinion of the investigator or healthcare team.

2.2.2 Benefit Summary

Participants will receive standard-of-care treatment with mFOLFOX6 chemotherapy and routine monitoring by clinical staff. [REDACTED] could improve clinical outcomes in combination with mFOLFOX6, though the likelihood of this outcome is unknown.

2.2.3 Overall Risk-Benefit Analysis

The increased risk to participants receiving [REDACTED] with mFOLFOX6 is perceived to be low compared to the risk of participants receiving mFOLFOX6 alone.

All participants receiving treatment with mFOLFOX6 with or without [REDACTED] are at risk of experiencing known and unknown complications and TRAEs associated with chemotherapy. Previous trials measuring the safety and tolerability of mAbs used to treat CLDN18.2-expressing GI cancers inform the dosing schedule and design of this trial to minimize the risk of TRAEs and hypersensitivities associated with [REDACTED] treatment.

Although the efficacy of [REDACTED] at preventing disease progression are currently unknown, the efficacy results of previous mAb trials support the hypothesis that [REDACTED] could confer clinical benefit to trial participants.

3 TRIAL OBJECTIVES AND ASSOCIATED ESTIMANDS

3.1 Primary Objective and Associated Estimands

The primary objectives of the study are to assess the safety and tolerability of single intravenous doses of [REDACTED] in combination with mFOLFOX6 and define the maximum tolerated dose or maximum feasible dose (MTD/MFD) and recommended phase II dose (RP2D) in subjects with advanced gastrointestinal (GI) adenocarcinoma.

The primary endpoints are:

- AEs, SAEs, TRAEs, clinical laboratory evaluations, vital signs, physical examinations, and electrocardiogram (ECG) results.
- Determination of the MTD, which is the highest dose where no more than 1 out of 6 possible subjects experience a DLT, or the MFD, which is the highest protocol-defined dose in the absence of exceeding the MTD.
- Determination of the RP2D, which is the highest dose with tolerable toxicities that may demonstrate anti-tumor activity with a clinically appropriate PK profile.

3.2 Secondary Objective and Associated Estimands

Although the antitumor activity of [REDACTED] has not been established, the treatment is intended to confer clinical benefit. Therefore, treatment efficacy will be evaluated in addition to safety and tolerability.

The secondary objectives of the study are to measure the pharmacokinetics (PK), immunogenicity, and anticancer therapeutic effect of [REDACTED] in combination with mFOLFOX6 in study participants over the course of 1 year.

The secondary endpoints are:

- PK parameters including area under the curve (AUC), maximum plasma concentration (C_{max}), time to maximum concentration (T_{max}), and drug half-life ($T_{1/2}$) evaluated measurements of circulating [REDACTED] in the blood plasma of participants collected prior to infusion; upon completion of infusion; 3, 6, 9, 12, and 24 hours post-infusion; and 2, 4, 7, 14, and 28 days post-infusion.
- Immunogenicity evaluated by measurements of anti-drug antibodies (ADA) in the sera of participants collected prior to infusion and 24 hours, 14 days, and 28 days post-infusion.
- Treatment efficacy determined by 52-week overall survival (OS) 52-week progression-free survival (PFS) assessed by iRECIST criteria.

3.3 Exploratory Objectives

The exploratory objectives of the study are to evaluate additional non-invasive measures of the antitumor activity of [REDACTED] in combination with mFOLFOX6.

The exploratory endpoints are:

- Biomarker response evaluated by measuring changes in circulating tumor DNA (ctDNA) levels measured from the blood plasma of participants collected prior to infusion and 2, 4, 8, 16, 26, 40, and 52 weeks post-infusion.
- Disease progression assessment via PET/CT imaging and overall response rate (ORR) with response including immune complete response (iCR) and immune partial response (iPR) rates as assessed by iRECIST criteria.

4 TRIAL DESIGN

4.1 Description of Trail Design

The study will follow the traditional 3+3 dose escalation study design with a toxicity-adjusted escalation plan. There will be up to five dose escalation cohorts starting with a 200 mg/m² initial dose of [REDACTED] (CD₀). The control group for the study will be placebo consisting of [REDACTED] excipient. Each cohort will consist of 4 participants randomly assigned to study drug or placebo at a 3:1 ratio. All investigational products will be administered in via a single intravenous (IV) injection over the course of 2 hours following site procedures.

If none of 3 participants randomized to [REDACTED] in the CD₀ cohort experience a DLT, the next cohort of four participants will be assigned at the next higher dose level of [REDACTED] (CD₁), which will be escalated by 100% D₀ (400 mg/m²). If the highest grade of toxicity was

grade 1 in the initial cohort, CD₁ will be escalated by 50% CD₀ (300 mg/m²). If the highest grade of toxicity was grade 2, CD₁ will be escalated by 25% CD₀ (250 mg/m²). If two or more of the participants receiving study drug experience a DLT (> 33%), no additional patients will be enrolled and CD₀ will be considered the maximum tolerated dose (MTD). If any one of the participants receiving study drug in CD₀ experiences a DLT (= 33%), four more participants will be enrolled and assigned at D₀. If two or more of the six participants (≥ 33%) receiving study drug at D₀ experience a DLT, no additional participants will be enrolled and D₀ will be considered MTD. If no additional patients in the expansion cohort experience a DLT, the next cohort of patients will be treated at D₁.

This dose escalation schema will continue up to five cohorts in the same manner through all five dose escalation cohorts. The maximum dose administered to the fifth cohort (D₄) will be 1000 mg/m² [REDACTED] is no toxicities are observed in participants randomized to [REDACTED].

Toxicity-Adjusted Dose Escalation Plan			
Cohort	No CD _{n-1} Toxicity	CD _{n-1} Grade 1 Toxicity	CD _{n-1} Grade 2 Toxicity
CD ₀	200 mg/m ²	-	-
CD ₁	400 mg/m ²	300 mg/m ²	250 mg/m ²
CD ₂	600 mg/m ²	500 mg/m ²	450 mg/m ²
CD ₃	800 mg/m ²	700 mg/m ²	750 mg/m ²
CD ₄	1000 mg/m ²	900 mg/m ²	850 mg/m ²

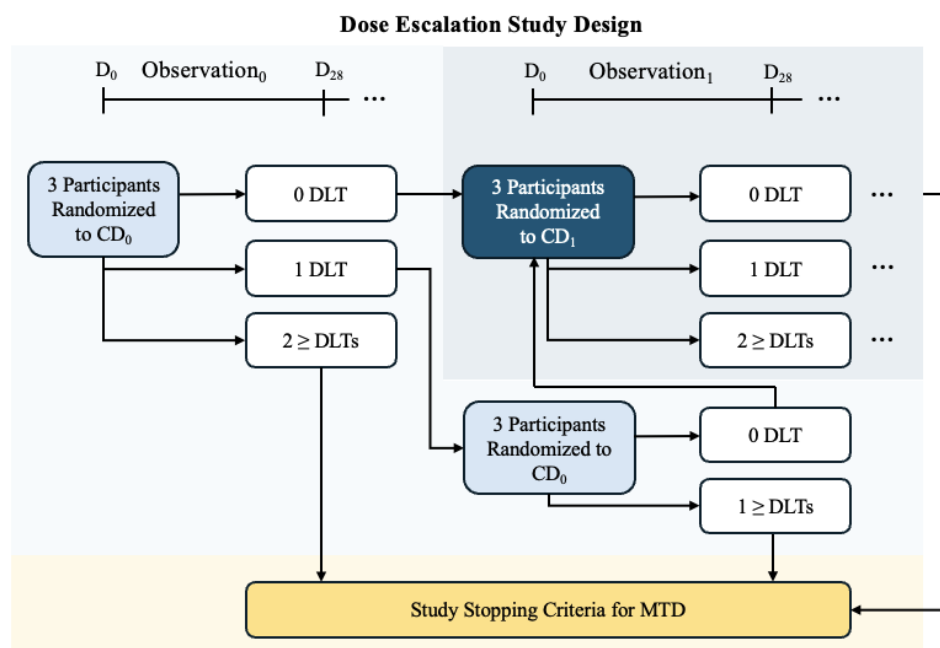


Figure 1: Schema of dose escalation rules and study stopping criteria based on observed dose limiting toxicities (DLTs) defined as defined as the occurrence of any AE $\geq$ grade 3 considered a TRAE possibility related to [REDACTED].

4.2 Rationale for Trail Design

The traditional 3+3 dose escalation is simple, easy to implement, enrolls an informative volume of participants, and has proven to have the ability to fulfill the main safety and tolerability objectives of phase I trials for anticancer therapies.^{17,18}

One of the potential disadvantageous of the 3+3 design is the risk of giving a large proportion of participants subtherapeutic levels of study drug. Because all study participants will be receiving a standard therapeutic dose of mFOLFOX6 in combination with all doses of [REDACTED], subtherapeutic dosing of the study drug places a conservative emphasis on patient safety without sacrificing appropriate clinical treatment. This design limits participant exposure to undue harm from overshooting the MTD.

4.2.1 Rationale for Intervention Model

Previous trials conducted with combination chemotherapy and monoclonal antibody infusions have been shown to improve treatment outcomes in phase II clinical trials. Should [REDACTED] prove to be an effective treatment in combination with chemotherapy, it could prove to be another effective treatment option for GI cancer patients.

4.2.2 Rationale for Control Type

Patients who are assigned to the control group will receive the standard-of-care treatment with mFOLFOX6. The placebo delivered will be the excipient of the [REDACTED] formulation, which is not predicted to be immunoreactive.

4.3 Trial Stopping Rules

No additional investigational products will be dosed if $2 \geq$ participants randomized to [REDACTED] in any single cohort experience DLTs within 28 days post-infusion. If DLTs are observed in $2 \geq$ participants randomized to [REDACTED] in any single cohort beyond the initial 28-day observation period and before the conclusion of the study, any scheduled investigational doses will continue to be administered but no additional participants will be enrolled.

The study will stop after the conclusion of all scheduled follow-up activities are completed for the participant who received the last dose of any investigational product or participants are lost to follow-up.

5 TRAIL POPULATION

5.1 Description of Trail Population and Rationale

The phase II FAST trial, which tested zolbetuximab in combination with chemotherapy (epirubicin, oxaliplatin, and capecitabine), found that the benefits of zolbetuximab were most prominent in the subgroup of patients with the highest percent ($\geq 70\%$) of malignant cells staining positive for CLDN18.2.¹⁹ Therefore, the trial population is restricted to patients with high-grade CLDN18.2 expression to maximize the potential benefits of [REDACTED] to trial participants.

CLDN18.2 acts downstream of HER2 and ER activity. Therefore, individuals who have received therapeutic intervention targeting/altering these pathways are excluded from participating in this study. Individuals who have received any HER2 anticancer treatment or hormone replacement therapy (HRT) will be excluded from the study based on contraindication.

GI cancers disproportionately affect older individuals, and the risk GI cancers increases with age.^{2,5} The median age of all patients with gastroesophageal adenocarcinomas is 68 years of age and 60% are ≥ 65 years of age.⁴ Therefore, elderly participants ≥ 65 years of ages are not excluded from the study provided they meet all inclusion criteria.

Pediatric participants and adults unable to consent for themselves will not be included in this study.

Biological sex and ethnicity are not factored into the inclusion or exclusion criteria. However, GI cancers such as GEJ adenocarcinoma are known to disproportionately affect black, Hispanic, and indigenous populations as well as biological females.⁶ Given these demographics, the study acknowledges that it may enroll a high distribution of participants from these backgrounds, which include historically marginalized populations (HMPs). Should this occur, the study would consider the participant population to be an accurate representation of the target clinical populations for [REDACTED] in combination with mFOLFOX6.

5.2 Inclusion Criteria

Participants must meet all the following criteria:

- Signed Informed Consent Form (IFC).
- ≥ 18 years of age.
- Biologically male or female.
- Refractory or recurrent histologically confirmed advanced adenocarcinoma in the stomach, esophagus, or gastroesophageal junction (GEJ) with immunohistochemically confirmed CLDN18.2 expression at a 3+ staining intensity or 2+ staining intensity in $\geq 40\%$ of tumor cells by a clinical pathologist.
- Measurable disease per Immune Response Evaluation Criteria in Solid Tumors (iRECIST).
- Adequate organ, coagulation, and hematologic function as defined in the protocol or have an eastern Cooperative Oncology Group (ECOG) performance status of ≤ 1 .

- Life expectancy of ≥ 3 months in the opinion of the investigator.
- For biological females of childbearing potential (5.4.1), a negative serum (blood) pregnancy test.
- Use of highly effective (5.4.2) non-hormonal birth control method(s) for a minimum of 60 days prior to consent and willingness to continue use for up to 12 months after receipt of the first study dose, or abstinence.

5.3 Exclusion Criteria

Any of the following would exclude a subject from participation in the study:

- Pregnancy or actively breastfeeding.
- Enrollment in another investigational study.
- Unresolved toxicities or adverse effects from previous anticancer therapies or other medications or treatments.
- Inadequate washout period for prior anticancer treatment prior to the first dose of the study drug as defined by the protocol.
- Receipt of therapy for HER-2 contraindications.
- Receipt of hormone replacement therapy (HRT).
- Receipt of any small molecule kinase inhibitor therapy.
- Active infection that requires systemic therapy within 3 weeks prior to the dose of investigational product.
- History of primary immunodeficiency or solid organ transplant.
- Positive human immunodeficiency virus (HIV-1) antibody result by history or at screening, or known positive for chronic or active hepatitis B or C or active hepatitis A.
- Use of systemic immunosuppressants or receipt of any monoclonal antibodies within 12 months prior to enrollment.
- Receipt of live, attenuated vaccine within 28 days prior to the dose of investigational product. (Note: Subject, if enrolled, should not receive live vaccine during the study).
- History of allergic reaction or hypersensitivity to other therapeutic antibodies or immunoglobulins.
- Any conditions that, in the opinion of the investigator or sponsor, would interfere with the evaluation of the investigational products, subject safety, or study results.

5.4 Contraception

5.4.1 Definitions Related to Childbearing Potential

Biological females of childbearing potential are defined as individuals who were clinically declared female at birth and are not surgically sterile (e.g. bilateral tubal ligation, bilateral oophorectomy, or complete hysterectomy) or postmenopausal. Postmenopausal is confirmed by follicle stimulating hormone (FSH) levels, which will be tested at the initial screening visit.

All participants who do not meet the criteria for childbearing potential will be considered participants of nonchildbearing potential.

5.4.2 Contraception Requirements

Highly effective non-hormonal methods of contraception include the following:

- Male condom plus spermicide
- Copper T intrauterine devices

Participants of childbearing potential must agree to abstinence or the use of either of these methods of contraception for a minimum of 60 days prior to consent and up to 12 months after receipt of the infusion of investigational product.

5.5 Lifestyle Restrictions

No restrictions on diet or activity are required.

5.6 Screen Failure and Rescreening

Subjects must meet all inclusion criteria with no exceptions. Subjects who fail screening due to failure to use of highly effective (5.4.2) non-hormonal birth control method(s) for a minimum of 60 days prior to consent yet who express willingness to continue use for up to 12 months after receipt of the first study dose may be rescreened for 60 days after the initial screening failure.

Subjects must not meet any exclusion criteria with no exceptions. Subjects who failure screening due to any of the following time sensitive exclusion criteria may be rescreened on or after the date at which they would no longer be excluded by the specific criteria:

- Inadequate washout period for prior anticancer treatment prior to the first dose of the study drug as defined by the protocol.
- Active infection that requires systemic therapy within 3 weeks prior to the first dose of investigational therapy.
- Use of systemic immunosuppressive agents or receipt of any monoclonal antibodies within 12 months prior to enrollment.

6 TRIAL INTERVENTION AND CONCOMITANT THERAPY

6.1 Description of Investigational Trial Intervention

The investigational trial intervention is [REDACTED] administered as single intravenous (IV) injection in combination with standard-of-care mFOLFOX6 chemotherapy. Participants randomized to the experimental arm will receive [REDACTED] in combination with mFOLFOX6 while participants randomized to the control arm will receive placebo (excipient only) with mFOLFOX6.

[REDACTED] is an anti-CLDN18.2 humanized IgG1 monoclonal antibody formulated for IV delivery. The placebo (excipient) used for the control arms of the trial is a non-active formulation manufactured through the same process as [REDACTED].

6.2 Rational for Investigational Trail Intervention Dose and Regimen

200 mg/m² is selected as the initial dose (CD₀) of [REDACTED] is determined based on the doses tested in a phase I study of anti-CLDN18.2 antibody IMAB362 (NCT00909025): Mild to moderate side events were first observed at 100 mg/m² and a dose range of 300 to 600 mg/m² was selected for further evaluation based on PK and PD data in this study.²⁰ A 200 mg/m² starting dose represents an informed and ethical balance between expected safety and efficacy.

The recommended mFOLFOX6 dosing regimen for this study is 85 mg/m² oxaliplatin and 400 mg/m² leucovorin delivered as a 2-hour IV infusion followed by 400 mg/m² 5-fluorouracil as a bolus and 2,400 mg/m² as a 46-hour infusion every 2 weeks. This mFOLFOX6 treatment regimen was used in a Phase IIb trial testing the efficacy of monoclonal antibody treatments in combination with mFOLFOX6 for metastatic colorectal cancer (NCT00609622).²¹

6.3 Investigational Trail Intervention Administration

Investigational product must be administered within 7 days after enrolled participants pass screening and on the same treatment cycle as mFOLFOX6. If administration is delayed, dosing should be rescheduled provided the 7-day window has not passed and mFOLFOX6 treatment is rescheduled for the same visit.

All investigational products will be administered in via a single intravenous (IV) injection over the course of 2 hours following site procedures. Investigation products should be administered during regular clinic hours and immediately prior to mFOLFOX6 infusions. No meal restrictions are required for infusion of investigational product unless otherwise indicated by the mFOLFOX6 regimen or site policy.

6.4 Investigational Trail Intervention Dose Modification

No dose modifications are planned or expected for [REDACTED] since it is delivered as a single injection. Doses will be escalated for randomized cohorts according to the previously defined toxicity-adjusted dose escalation rules (4.1).

mFOLFOX6 treatment regimens are considered appropriate as part of the standard care of patients with advanced gastrointestinal adenocarcinomas.^{11,21,22} Deviations to the recommended

mFOLXFOL6 dosing regimen for this study are accepted if recommended in the professional opinion of the healthcare team of the participant and recorded in all source documentation.

6.5 Management of Investigational Trial Intervention Overdose

Participants will be monitored closely for 1 hour after IV infusion of investigational product. In the event of suspected overdose, appropriate supportive care will be provided, and any overdose will be documented and reported as an adverse event.

6.6 Preparation, Storage, Handling and Accountability of Investigational Trial Intervention

6.6.1 Preparation of Investigational Trial Intervention

Investigational product will be aseptically prepared in an International Organization for Standardization (ISO) Class 5 environment. Finished product must undergo sterility and endotoxin testing prior to administration. Sterility tests must be performed in accordance with United States Pharmacopeia (USP) <1111> and product must meet the defined standards. Endotoxin test measurements must ensure that the endotoxin units (EU) delivered per kilogram (kg) of body weight per hour (hr) is < 5 EU/kg/hr in accordance with USP <85>.

6.6.2 Storage and Handling of Investigational Trial Intervention

Investigational products will be stored in accordance with IHC GCP in locations equipped with continuous temperature and humidity monitors. Records of receipt, storage, dispensing, and disposal will be documented in the electronic trail master file (eTMF).

6.6.3 Accountability of Investigational Trial Intervention

Investigational product will be distributed to the site for administration after confirmation of participant enrollment and scheduled visit. Partially used product will be disposed of following standard site procedures. Unused product will be returned to storage. All disposal and reconciliation will be documented in the eTMF.

6.7 Investigational Trial Intervention Assignment, Randomization and Blinding

6.7.1 Participant Assignment to Investigational Trial Intervention

Participants are enrolled in the study once written consent is obtained via a completed informed consent form (ICF). Each enrolled participant will be assigned a subject identification (SID) number assigned by the central data system and screening evaluations will be performed to assess study eligibility based on exclusion/inclusion criteria.

Participants who fail to meet the inclusion/exclusion criteria will not participate in any subsequent study procedures and will not receive investigational product.

A log of all enrolled participants and the source documentation of all screening procedures will be maintained in study files regardless of screening outcome. The documentation for all screening failures and reason(s) for screening failures will be included.

6.7.2 Randomization

Participants will be randomly assigned to study drug or placebo at a 3:1 ratio for each dose-escalation cohort. Randomization assignments will be implemented through the IXRS®³ interactive response technology platform.

6.7.3 Measures to Maintain Blinding

To maintain the integrity of the single-blind study, all study personnel will be instructed not to discuss any information related to treatment assignment except the minimum information necessary to conduct study procedures. The SID will be used to identify the participant through all phases of the study and will be indicated on all source documentation and data associated with the participant to enable anonymization or deidentification.

6.8 Investigational Trial Intervention Adherence

Investigational products are administered on-site as single IV injections under direct clinical supervision and will be documented in the source documents and electronic case report form (eCRF).

6.9 Description of Noninvestigational Trial Intervention

The auxiliary medicinal product used in this study is mFOLFOX6. It is a combination chemotherapy consisting of modified folinic acid (leucovorin, calcium folinate), 5-fluorouracil (5-FU), and oxaliplatin (L-OHP). mFOLFOX6 is typically administered intravenously every 2 weeks in the hospital or clinic with 5-fluorouracil infusions continued at home via a portable pump for 46 hours.

6.10 Concomitant Therapy

Any concomitant therapy that interferes with study objectives will result in participant discontinuation (7.1) or withdrawal (7.2) from administration of the investigational product.

7 PARTICIPANT DISCONTINUATION OF TRIAL INTERVENTION AND DISCONTINUATION OR WITHDRAWAL FROM TRIAL

7.1 Discontinuation of Trial Intervention for Individual Participants

7.1.1 Permanent Discontinuation of Trial Intervention

A participant will be permanently discontinued from administration of the randomized investigational product ( or placebo) in any of the following circumstances:

- Development of a dose-limiting toxicity (DLT) or other severe adverse event (SAE) that poses an unacceptable risk to the participant in the professional judgement of the investigator of healthcare team.
- Measurable disease progression determined per iRECIST criteria.
- Clinical deterioration of organ, coagulation, and hematologic function and/or to an ECOG performance status of ≥ 1 .
- Voluntary withdrawal of consent where the participant no longer wishes to receive trial intervention.
- Requirement of a concomitant treatment or therapy that interferes with study objectives.
- Pregnancy or other protocol noncompliance that significantly impacts study objectives or integrity.

Because this is a study of a single infusion of investigational product given in combination with mFOLFOX6 chemotherapy, a participant may also be permanently discontinued from the trial if they will no longer receive treatment with mFOLFOX6 for any of the following circumstances:

- Investigator or healthcare team determine that the continued administration of mFOLFOX6 is medically inappropriate standard of care.
- Participant withdraws consent to receive continued administration of mFOLFOX6.

7.1.2 Temporary Discontinuation of Trial Intervention

Because the investigational product (██████████ or placebo) is given as a single infusion, temporary discontinuation of administration does not apply. Administration of investigational product may be delayed per local site operational procedures and sponsor direction.

Temporary interruptions, dose modifications, or delays to the administration of mFOLFOX6 may be applied during the study appropriate to the medical needs of the participant.

All interruptions and rationales will be documented in the source documents and electronic case report form (eCRF).

7.1.3 Rechallenge

Rechallenge is not allowed within the scope of this study.

7.2 Participant Discontinuation or Withdrawal from the Trial

Participants will be withdrawn from continued monitoring and data collection in any of the following circumstances:

- Death or transition to hospice or palliative care where further study participation is deemed inappropriate.
- Requirement of a concomitant treatment or therapy that interferes with study objectives.

- Pregnancy or other protocol noncompliance that significantly impacts study objectives or integrity.
- Considered lost to follow-up (7.3).

Participants may voluntarily discontinue or withdraw consent for study-related procedures and data collection at any point during the study. In the event of voluntary discontinuation or withdrawal, the study team will proceed as follows:

1. Attempt to determine the reason for withdrawal if the participant is willing to provide it and if the withdrawal is partial (agreement to some procedures) or complete (withdrawal from all follow-ups, data collection, and trial procedures).
2. Ask the participant if their previously collected data may still be used for analysis. Previously collected data will be retained for analysis unless specifically requested for selection by the participant.

The reason for discontinuation or withdrawal will be documented in the source documents and electronic case report form (eCRF) or else marked unknown.

7.3 Management of Loss to Follow-Up

A participant will be considered lost to follow-up only after multiple unsuccessful attempts to establish contact through at least 3 distinct methods of communication.

If a participant misses a scheduled visit or does not respond to routine communication throughout the course of the study, the following attempts to establish contact will be made, in the following order:

1. First attempt: Contact the participant via their preferred method of communication (e.g. telephone, text, or email) within 24 hours of a missed scheduled visit.
2. Second and third attempts: Contact the participant via their preferred method of communication and a second communication modality (e.g. text and email) over the following 7 days after a missed scheduled visit.
3. Final attempt: Contact the designated secondary or emergency contact of the participant after 3 unsuccessful attempts to establish communication within 7 days of a missed scheduled visit. Additionally, send written communication via mail to request contact and offer support.

If all the following attempts have been made and the contact with the participant is not reestablished within 28 days of the missed scheduled visit, the participant will be considered lost to follow-up.

If the participant reestablishes contact after being considered lost to follow-up, study personnel will reschedule any remaining protocol-specified follow-up activities according to the study schedule (1.3). The study team will ask the participant about any events that may qualify as an AE, SAE, TRAE, or DLT that may have occurred while the participant was lost to follow-up.

8 TRIAL ASSESSMENTS AND PROCEDURES

8.1 Trial Assessments and Procedures Considerations

All trial assessments and procedures will be conducted following ICH GCP guidelines and will be subject to monitoring to ensure participant safety.

Trial assessments should be completed as indicated in the schedule of activities. Subject safety will take precedence over trial assessments and procedures, which may be delayed, modified, or omitted based on the clinical judgement of the investigator or healthcare team of the participant. All results, delays, or modifications will be documented in the source documentation and eCRFs.

8.2 Screening/Baseline Assessments and Procedures

The following assessments and procedures are required during screening to confirm eligibility:

- ECOG performance evaluations.
- Physical examinations (8.4.1).
- Serum tests for pregnancy (8.4.5) for biological females of childbearing potential (5.4.1) or FSH to confirm the postmenopausal status of non-menstruating biological females (8.4.4), as appropriate.

ECOG performance status:

Performance status will be assessed at screening in tandem with physical examinations to describe the subject's level of functioning in terms of their ability to care for themselves, daily activity, and physical ability. Status will be determined according to the criteria defined by the Eastern Cooperative Oncology Group (ECOG) performance scale²³ as reproduced below:

Grade	ECOG Performance Status
0	Fully active, able to carry on all pre-disease performance without restriction
1	Restricted in physically strenuous activity but ambulatory and able to carry out work of a light or sedentary nature, e.g., light housework, office work
2	Ambulatory and capable of all selfcare but unable to carry out any work activities; up and about more than 50% of waking hours
3	Capable of only limited selfcare; confined to bed or chair more than 50% of waking hours
4	Completely disabled; cannot carry on any selfcare; totally confined to bed or chair
5	Dead

Qualified clinicians will determine the ECOG performance status of subjects using their professional judgement informed by patient interviews and observation. ECOG may be reevaluated and updated for participants as clinically indicated; in these cases, the ECOG status reported at screening will serve as the baseline metric.

8.3 Efficacy Assessments and Procedures

Tumor response will be assessed by iRECIST criteria for target lesions assessed by PET/CT.

Positron emission tomography/computed tomography (PET/CT):

PET/CT scans will be performed at as indicated in the schedule of events or as determined clinically appropriate to assess metabolic tumor activity, monitor treatment efficacy, and/or confirm radiographic response or progression.

Participants will be instructed to fast for ≥ 6 hours prior to radiotracer administration. PET images will be acquired from the skull base to mid-thigh or as clinically indicated for the participant. PET imaging and CT acquisition parameters will follow site-specific guidelines.

Categorization of tumor lesions:

- **Measurable:** Lesions that can be accurately measured in at least one dimension ≥ 10 mm by CT or caliper measurement (when superficial). The longest axis will be recorded. Lymph nodes used as target lesions must be ≥ 15 mm in short axis.
- **Non-measurable:** All lesions non defined as measurable. Truly non-measurable lesions include leptomeningeal disease, ascites, pleural/pericardial effusion, and inflammatory breast disease, lymphangitic involvement of skin or lung, abdominal masses/abdominal organomegaly identified by physical exam that is not measurable by reproducible imaging techniques.
- **Target lesions:** Measurable lesions up to a maximum of 5 lesions (2 per organ).
- **Non-target lesions:** on-measurable lesions, including multiple non-measurable lesions in the same organ, and all measurable lesions not considered target lesions.

Target lesions should be chosen at baseline and should be reported in all subsequent imaging results, even if they become non-measurable.

Evaluation of target lesions:

The following tumor response categories will be used for target lesions as defined in above:

- **Immune complete response (iCR):** Disappearance of all target lesions. Any pathological lymph nodes (whether target or non-target) must have reduction in short axis to <10 mm.
- **Immune partial response (iPR):** At least a 30% decrease in the sum of target lesion diameters compared to baseline.
- **Immune stable disease (SD):** Neither sufficient shrinkage for iPR nor sufficient increase for iUPD/iCPD.
- **Immune unconfirmed progressive disease (iUPD):** At least a 20% relative and 5 mm absolute increase of sum of target lesion diameters compared to the smallest sum of target lesions diameters ever recorded for the participant, progression of non-target lesions, or appearance of new lesions.

- **Immune confirmed progressive diseases (iCPD):** Continued progression of the iUPD scan with progressive disease that meets all the criteria of iUPD.

iUPD will be determined from the PET/CT scan scheduled 4 weeks after infusion with investigational product and iCPD will be determined from all subsequent PET/CT scans as outlined in the schedule of events. iRECIST differentiates between iUPD and iCPD because initial lesion growth due to immunotherapies may be due to immune-related inflammation.^{24,25}

Evaluation of non-target lesions:

Non-target lesions will be assessed qualitatively for absent, present, or unequivocal progression. Unequivocal progression of existing non-target lesions is defined as an overall substantial worsening in non-target disease such that, even in the presence of iPR or iSD in target disease, the overall tumor burden has increased sufficiently to merit discontinuation of therapy.²⁶

The following tumor response categories will be used for non-target lesions as defined in above:

- **Immune complete response (iCR):** Disappearance or absence of all non-target lesions. All lymph nodes must have reduction in short axis to <10 mm.
- **Immune non-complete/partial response (non-iCR/non-iPR):** Persistence or presence of one or more non-target lesion(s).
- **Immune unconfirmed/confirmed progressive disease (iUPD/iCPD):** Appearance of one or more new lesions or unequivocal progression of existing non-target lesions. iUPD will be assessed at PET/CT 4 weeks post-infusion and iCPD at subsequent timepoints.

Evaluation of overall response:

Overall response will be determined based on the evaluation of target, non-target, and new lesions as follows:

Target lesions	Non-target lesions	New lesions	Overall response
iCR	iCR	No	Complete response
iCR	Not evaluable	No	Partial response
iCR	Non-iCR/non-iPR; non-iUPD/non-iCPD	No	Partial response
iPR	Non-iUPD/non-iCPD and not evaluable	No	Partial response
iSD	Non-iUPD/non-iCPD and not evaluable	No	Stable disease
Not all evaluated	Non-iUPD/non-iCPD	No	Not evaluable
iUPD/iCPD	Any	Yes or No	Progressive disease
Any	iUPD/iCPD	Yes or No	Progressive disease
Any	Any	Yes	Progressive disease

Not evaluable is defined as either when no or only a subset of lesions measurements are made at an assessment.

Overall response rate (ORR) will be calculated as a percentage representing the proportion of patients who respond to treatment: the sum of the number of participants achieving an overall complete response (CR) or partial response (PR) will be divided by the total number of evaluable participants, multiplied by 100.

8.4 Safety Assessments and Procedures

8.4.1 Physical Examinations

Physical examinations will be performed according to institutional guidelines during screening. These will include clinical assessments of the head, eyes, ears, nose, and throat; respiratory, cardiovascular, gastrointestinal (GI), urogenital, musculoskeletal, neurological, psychiatric, dermatological, hematologic/lymphatic, and endocrine systems; weight; and height.

8.4.2 Vital Signs

Vital signs including temperature, blood pressure (BP), pulse rate, respiratory rate, and weight will be measured as outlined in the schedule of activities.

8.4.3 Electrocardiograms

A 12-lead ECG will be recorded where indicated in the schedule of activities and electrode positions will follow the standard 12-lead placement. Date and time settings will be checked before ECG collection and aligned with an official timekeeper for all machines used in the study. The same recorder will be used for each subject at each time point whenever possible.

Heart rate, RD, RR, QRS, and QT intervals from the primary lead will be reported. Lead V2 will be analyzed and reported as the primary lead; if analysis of lead V2 is impossible, lead V5 will be analyzed as reported as the primary lead.

In case of clinically significant ECG abnormalities, 2 additional 12-lead ECGs should be obtained over a brief period (e.g. 30 minutes) to confirm prolongation.

8.4.4 Clinical Laboratory Assessments

The clinical laboratory tests that will be performed for this study are outlined in section 12.1. Procedures for collecting, processing, storing, and shipping samples are outlined in the Laboratory Manual provided to the study site. All clinical laboratory tests will be performed in a Clinical Laboratory Improvement Amendments (CLIA)-certified laboratory.

Serum follicle-stimulating hormone (FSH) testing will be performed during screening to confirm the postmenopausal status of non-menstruating biological females. A participant will be considered postmenopausal if the serum FSH concentration is between 23.0 - 116.3 mIU/mL or the postmenopausal reference range otherwise defined by the laboratory.

The investigator or designated clinical laboratory team will review all laboratory results for significant abnormalities which will be recorded and reported as adverse events following the study protocol (9.1). Repeat testing of abnormal results may be performed if deemed appropriate by the clinical judgement of the study team and will occur as soon as possible (preferably within

24 to 48 hours). The investigator will categorize all abnormal laboratory results as either not clinically significant (NCS) or clinically significant (CS), including those reported as AEs (9.1.1).

8.4.5 Pregnancy Testing

Serum pregnancy testing will be performed in a CLIA-certified laboratory during screening for participants of childbearing potential (5.4.1). A negative serum pregnancy test is defined as a beta-human chorionic gonadotropin (β -hCG) level below the lower limit of detection at the laboratory. Tests with abnormal results will be repeated as soon as possible (preferably within 24 to 48 hours).

8.5 Pharmacokinetics

PK parameters will be calculated from measurements of circulating [REDACTED] in the post-infusion blood plasma of participants collected during the visits indicated in the schedule of events to adequately characterize the distribution and elimination phases of [REDACTED]. The methods for calculation are described in the separate statistical analysis plan (SAP) developed for this study. Parameters that will be calculated include area under the curve (AUC), maximum plasma concentration ($C_{\max}$), time to maximum concentration ($T_{\max}$), and drug half-life ($T_{1/2}$). The results from the analysis will be presented by participant and summarized by cohort and study day, including absolute values and changes from baseline.

8.6 Biomarkers

8.6.1 Genetics and Pharmacogenomics

Blood plasma samples will be collected as specified in the schedule of activities. Procedures for isolating and analyzing cell-free DNA (cfDNA) are outlined in the Laboratory Manual provided at the study site.

Circulating tumor DNA (ctDNA) will be detected using next-generation sequencing (NGS) or digital polymerase chain reaction (dPCR). Analyses may include quantification of:

- Total ctDNA concentration.
- Change in ctDNA concentration over time, including percent change from baseline.
- Tumor-associated genetic measurements, including variant allele frequency (VAF); identification of single nucleotide variants (SNVs), insertions/deletions, copy number alterations, or rearrangements; and emergence of mutations potentially associated with treatment resistance.

Blood plasma and plasma-derived DNA samples may be stored for future exploratory analyses related to the objectives of the study consistent with the duration outlined in the ICF. Genetic data will be handled in accordance with the Health Insurance Portability and Accountability Act (HIPAA) regulations, the International Council for Harmonization E18 Guidance for Industry (ICH E18), and all applicable data protection standards (11.8).

Genetic biomarker results will not be used to guide participant treatment decisions.

8.6.2 Pharmacodynamic Biomarkers

Not applicable.

8.7 Immunogenicity Assessments

Anti-drug antibodies (ADA) will be assessed periodically to evaluate potential immune responses against [REDACTED] from serum samples collected at the timepoints indicated in the schedule of events.²⁷ Immunogenicity testing will follow a multi-tiered strategy as follows:

- Tier 1: A bridging enzyme-linked immunosorbent assay (ELISA) will be used to detect if ADAs are present.
- Tier 2: Competitive inhibition assays will be used to confirm specificity.
- Tier 3: Titering assays will be performed to quantify the magnitude of the ADA response and neutralizing assays to determine whether ADAs inhibit the biological activity of [REDACTED]. Optional risk-based characterizations of cross reactivity to endogenous proteins, isotype assignment, and epitope specificity will be performed as appropriate.

8.8 Medical Resource Utilization and Health Economics

Not applicable.

9 ADVERSE EVENTS, SERIOUS ADVERSE EVENTS, PRODUCT COMPLAINTS, PREGNANCY AND POSTPARTUM INFORMATION, AND SPECIAL SAFETY SITUATIONS

9.1 Definitions

9.1.1 Definitions of Adverse Events

An adverse event (AE) is defined by the ICH E2A as any untoward medical occurrence in a patient or clinical investigation subject administered a pharmaceutical product and which does not necessarily have to have a causal relationship with the treatment. An adverse event (AE) can therefore be any unfavorable and unintended sign (including an abnormal laboratory finding, for example), symptom, or disease temporally associated with the use of a medicinal product, whether considered related to the investigational product or not. AEs are considered treatment-related adverse events (TRAEs) if there is a possibility that they are related to the investigational product. The investigator will determine on a case-by-case basis if an AE qualifies as a TRAE based on temporal association, biological plausibility, and alternative etiologies.

All AEs will be graded according to the Common Terminology Criteria for Adverse Events (CTCAE) v6.0.¹ For any term that is not specifically listed, severity must be assigned a grade of 1 through 5 using the general CTCAE v6.0 guidelines, reproduced below:

Grade 1	Mild; asymptomatic or mild symptoms; clinical diagnostic observations only; intervention not indicated.
Grade 2	Moderate; minimal, local or noninvasive intervention indicated; limiting instrumental activities of daily living (ADL).
Grade 3	Severe or medically significant but not immediately life-threatening; hospitalization or prolongation of hospitalization indicated; disabling; limiting self-care ADL.
Grade 4	Life-threatening consequences; urgent intervention indicated.
Grade 5	Death related to AE.

An abnormal laboratory test not already associated with an AE will be recorded as an AE if it fulfills all the following criteria:

- A repeat test has been performed to confirm the abnormal result.
- An intervention for management of the abnormal result is required.
- The investigator considered the abnormal results to be an AE.

9.1.2 Definitions of Serious Adverse Events

A serious adverse event (SAE) is defined as any AE that results in death, is immediately life-threatening, requires inpatient hospitalization or prolongation of existing hospitalization, or results in persistent or significant disability/incapacity. Any important medical event that may require medical intervention to prevent one of the outcomes listed previously or may otherwise jeopardize the participant will also be classified as a SAE. The medical or scientific judgement of investigators or healthcare team of the participants should be exercised on deciding whether an event constitutes a SAE under this definition.

SAEs will be reported within 24 hours from the time the investigator is made aware of the SAE.

9.1.3 Definitions of Product Complaints

A complaint is defined by 21 CFR 820.3(b) as any written, electronic, or oral communication that alleges deficiencies related to the identity, quality, durability, reliability, safety, effectiveness, or performance of a device. The study will use the same definition for products.

9.2 Timing and Procedures for Collecting and Reporting

9.2.1 Timing

AEs and SAEs will be collected from the time of informed consent signature through the end of the follow-up period for the participant or conclusion of the study, whichever comes first.

AEs will be monitored until the AE has resolved (or stabilized, for abnormal clinical laboratory results) or the participant completes their last assessment/visit, is lost to follow-up, or has died, whichever is earliest. Any AEs that are unresolved by the last follow up visit of the participant may be followed by the investigator for as long as medically indicated but do not require further

reporting in the eCRF (9.2.3). TRAEs and SAEs will be continually monitored and recorded in the eCRF for as long as the study team is able to follow the participant for safety.

9.2.2 Collection Procedures

AEs and SAEs will be assessed by direct observation and participant assessments/visits or upon notification. Product complaints will be collected through the reporting channels appropriate to the product following site procedures.

9.2.3 Reporting

The investigator or designee(s) will record and/or report an AE or SAE within 24 hours from the time the investigator is made aware of the AE or SAE. AEs will be recorded on the source documents and eCRF using the recognized medical term or diagnosis that accurately reflects the events. If the AE meets the criteria of an SAE, the investigator or designee(s) must fax or email a completed or **SAE Report Form** to the sponsor using the number or email provided on the form. SAEs and follow up information will be reported to the IRB according to the policies of the IRB.

9.2.4 Adverse Events of Special Interest

An adverse event of special interest (AESI) is defined by ICH E19 as an event (serious or non-serious) of scientific or medical concern specific to the product or program, for which ongoing monitoring and rapid communication by the investigator to the sponsor can be appropriate.

AESIs for [REDACTED] include hypersensitivities and severe infusion reactions that resemble anaphylaxis associated with monoclonal antibody therapies (mAbs).^{15,16}

Hypersensitivities – Hypersensitivities are exaggerated and/or pathological immune responses to exposure to a novel antigen that are generally unrelated to pharmacological activity. There are 4 categories of hypersensitivities defined by their mechanism of action, all of which have been observed with mAbs therapies and are considered AESI for [REDACTED]. The investigator and study team are not required to classify hypersensitivities by type, though examples are listed below to help the investigator and study team to appropriately classify reactions as true hypersensitivities and AESIs according to their professional judgement.

Type	Description	Symptoms
1	IgE antibody-mediated reactions	anaphylaxis, urticaria
2	Cytotoxic reactions	hemolytic anemia, early-onset neutropenia
3	Immune-complex-mediated hypersensitivities	serum sickness, pneumonitis
4	Delayed, cell-mediated responses	Steven-Johnson syndrome (SJS), toxic epidermal necrolysis (TEM)

Infusion reactions – Infusion reactions are common during first exposures and often involved mild to moderate flu-like symptoms. However, infusion reactions can be serious or fatal and may resemble anaphylaxis. Anaphylaxis is a severe and sudden reaction to infusion typically including severe systemic skin and/or mucosal reactions which often occur after repeat exposures to an antigen. Serious infusion reactions and anaphylaxis may be difficult to distinguish. The

investigator and study team should classify reactions occurring shortly after infusions according to their professional judgement and according to the National Institute of Allergy and Infectious Diseases (NIAID) and Food Allergy and Anaphylaxis Network (FAAN) guidance for anaphylaxis diagnosis.²⁸

The investigator or designee(s) must report the AESI within 24 hours from the time the investigator is made aware of the AESI by faxing or emailing a completed Safety Report Form to the sponsor using the number or email provided on the form. AEs and SAEs that meet the criteria of an AESI will be flagged as such in the eCRF.

9.2.5 Disease-related Events or Outcome Not Qualifying as AEs or SAEs.

Not applicable.

9.3 Pregnancy and Postpartum Information

Pregnancies in female participants who have received investigational product must be immediately reported to the healthcare team and study team. Pregnant participants will be withdrawn from the study within 24 hours. Pregnancies in a partner of a male participant who has received investigation product should also be immediately reported, though the participant will not be withdrawn from the study.

The investigator or designee(s) will report all pregnancies within 24 hours from the time the investigator is made aware of the pregnancy. The investigator must fax or email a completed Safety Report Form to the sponsor using the number or email provided on the form.

If possible, pregnancies and their outcomes will be documented in the source documents and eCRF. The study must obtain consent of the partner of the participant prior to obtaining information about the pregnancy using an IRB/IEC approved consent form. Postpartum information may include but is not limited to:

- Pregnancy outcome, date of delivery or termination, mode of delivery.
- Maternal complications during delivery or postpartum, SAEs occurring postpartum, and outcome of any pregnancy-related AEs.
- Infant biological sex, weight, length, gestational age at birth, neonatal complications, and presence or absence of congenital anomalies or birth defects.

A pregnancy in a female participant or partner of a male participant is neither an AE or SAE, unless a complication relating to the pregnancy occurs that may qualify as an AE or ASE. A congenital anomaly or birth defect in the offspring of a participant will be considered a SAE and will be reported using the SAE Report Form.

10 STATISTICAL CONSIDERATIONS

10.1 General Considerations

A separate statistical analysis plan (SAP) exists for this study. The analysis will include all participant data at trial completion.

11 TRIAL OVERSIGHT AND OTHER GENERAL CONSIDERATIONS

11.1 Regulatory and Ethical Considerations

The study will be performed in accordance with ethical principles that have their origin in the Declaration of Helsinki and are consistent with International Council for Harmonization Good Clinical Practice (ICH/GCP) guidelines, and all applicable regulatory requirements.

The study may enroll participants from vulnerable populations including individuals from underprivileged socio-economic backgrounds, non-English speaking individuals or individuals with limited English proficiency, and elderly individuals. To ensure equitable access to the study site and protection of patient rights, the following practices must be implemented:

- Transportation support or reimbursement is offered where applicable to reduce financial barrier to study participation.
- Information is given to participants and their families or caregivers in a language they can understand. Translations must be approved by an IRB and other regulatory bodies.
- Periodic review and ethical oversight by the IRB/IEC to ensure fairness and equity.

11.2 Trial Oversight

The IRB and IEC must approve the study protocol, site study conduct, informed consent form (ICF), and any other written information provided to the participants including advertisements that will be used for the study. The IRB and IEC will be permitted to review the portions of participant medical records directly related to the study. Participants will be informed that their medical records may be reviewed in this manner in their ICF.

11.2.1 Investigator Responsibilities

The Investigator is responsible for ensuring participant safety, ethical conduct, protocol compliance, data integrity and protections, and reporting activities. The investigator will:

- Obtain written informed consent from each subject before participation in the trial.
- Assess the clinical significance of all abnormal laboratory values as defined by appropriate reference range(s).
- Report all AEs and SAEs and determine the causality of all AEs and TRAEs.
- Provide support for all monitoring and auditing procedures including direct access to documentation guaranteed by the study (11.6).
- Continually identify and promptly report protocol deviations to the IRB, IEC, and DSMB.

11.2.2 Sponsor Responsibilities

The sponsor is responsible for overseeing study progress as well as adherence to all applicable ICH GCP guidelines and local regulations. The sponsor will:

- Ensure the study is designed, conducted, documented, and reported in accordance with all applicable regulations.
- Secure approval of the clinical study protocol, informed consent form, and any additional study documents by the IRB/IEC prior to study initiation.
- Report all AEs that occur during the study to the FDA, IRB, IRC, DMSB, other regulatory bodies as applicable and appropriate.
- Oversee quality assurance representative(s) performing audits and site visits (11.6).
- Promptly inform the IRB/IEC/DSMB and any appropriate regulatory bodies of any significant developments and findings.

11.3 Informed Consent Process

The investigator and clinical study teams will ensure all the following conditions are met for each participant before their formal enrollment in the study:

- Oral and written information about the nature, purpose, and possible risks and benefits of the study is given in clear and accessible language.
- Sufficient time and opportunity to ask questions and consider the information provided to them is given.
- Any incentives for the participants in the study as well as provisions for participants harmed as a consequence of start participation are described in the informed consent form (ICF) that is approved by an IRB/IEC and shared with the participant.
- Explicit notice that the participant may voluntarily discontinue or withdraw from the study at any time without penalty or loss of medical care.
- Participant has signed and dated an ICF before participating in any study procedure.
- The original, signed ICF are securely stored in the source documents.
- A copy of the signed ICF is given to the participant.

The investigator and clinical study teams must re consent all active study participants using the same process as the initial informed consent if any future protocol amendments affect the ICF.

11.4 Committees

An independent Data Safety Monitoring Board (DSMB) will review and evaluate accumulated study data for participant safety, study conduct and progress, and, when appropriate, efficacy. The DSMB may make recommendations for trial modification, amendment, or termination based

on their overall risk-benefit assessment of the study data. The DSMB is responsible for internally determining its review process prior to initiating any data review.

11.5 Insurance and Indemnity

The arrangements for participant insurance and indemnity are addressed in the clinical trial agreement (CTA). Coverage includes injury or harm to participants that is directly attributable to their participation in the study to the extent required by the law.

11.6 Risk-Based Quality Management

A separate Quality Management Plan (QAP) exists for this study. Quality control activities will be conducted according to internal site procedures. The following oversight procedures will be managed by the sponsor:

Monitoring: Study monitors, including quality assurance representatives of the sponsor, may contact and visit the investigator and will be permitted to inspect all source documents, eCRFs, and other study documentation upon request, provided patient confidentiality is maintained and the inspection is conducted in accordance with local regulations. All necessary data and documents will be made available for inspection along with any required study staff.

Audits: The IRB/IEC, regulatory authorities, of quality assurance representative of the sponsor may request access to all source documents, eCRFs, and other study documentation for audits or inspections. The investigator will ensure direct access to these documents.

11.7 Data Governance

Key processes enabling accurate collection, reporting, monitoring, transfer, retention, and access to study data are detailed in the Data Management Plan (DMP) and overseen by the DSMB.

11.8 Data Protection

All data will be handled in compliance with the Health Insurance Portability and Accountability Act (HIPAA) regulations, the International Council for Harmonization E18 Guidance for Industry (ICH E18), the General Data Protection Regulation (GDPR), and all applicable data privacy laws. Technical specifications are details in the DMP.

Participant confidentiality will be maintained by all the following conditions:

- Participants will be assigned unique study identification numbers for deidentification.
- Electronic records will be encrypted and secured in password protected databases. Paper records will be stored in secure, access-controlled locations.
- Clinical data derived from source documents will be entered in a 21 CFR Part 11-compliant electronic data capture (EDC) system at clinical sites. The data system will include password-protected access, backups, and an audit trail.

- Access to records will be limited to only authorized study personnel that need access to perform essential activities.
- Data shared with regulatory agencies, sponsors, or other third parties (11.12) will be anonymized or deidentified.

The DSMB will periodically review the confidentiality of all accumulated study data and ensure the above conditions are maintained through the study duration.

11.9 Source Data

Source data include any worksheets, records, memoranda, and notes of original and firsthand observations and activities that are necessary for the reconstruction and evaluation of the study. Source documentation and data must be attributable, legible, contemporaneous, original, accurate, and complete following IHC GCP and FDA guidelines for Good Documentation Practices (GDP).

Investigator Expectations for Source Data: All data recorded in eCRFs must be supported by source documentation and traceable to the original source. Clinical data derived from source documents will be entered an electronic data capture (EDC) system. Data not captured in the EDC or eCRFs will be received and stored as external files in secured databases. Internal and quality checks will be periodically performed to identify data that appear inconsistent, incomplete, or inaccurate.

Trial Monitor Expectations for Source Data: Data entered in any eCRF/EDC will be verified relative to source documentation to ensure the reliability of trial results and protection of trial participants. Additionally, data retention practices must follow ICH guidelines which provide that essential documents must be retained for a minimum of 2 years after the last approval of a marketing application in an ICH region and until there are no pending or contemplated marketing applications in an ICH region or at least 2 years have elapsed since the formal discontinuation of clinical development of the investigational product. Documents must be retained for a longer period if required by the applicable local requirements.

11.10 Protocol Deviations

Study procedures will not be changed without being documented in a study protocol amendment and approval of the amendment by the relevant IRB/IEC and, if applicable, the national regulatory authority. Any approved subsequent amendments will be distributed to the principal investigator and study teams before implementations.

11.11 Early Site Closure

The sponsor and investigator retain the right to initiate site closure before the study is scheduled to end. The study site(s) may be closed early due to any of the following conditions:

- Early data show serious harm or unexpected toxicity such that the risks of treatment with  outweigh the estimated benefits as described in the protocol.

- The trial is unable to enroll enough participants for the dose escalation study.
- Sponsor withdrawal, investigator retirement, or lack of resources or interest.
- Approval of competing therapies makes the study irrelevant.
- Suspension or termination by an IRB or other regulatory agency.

If the study terminated early due to site closure, all study participants will be promptly informed and will continue to receive medical care not specific to the study. The study team and healthcare providers will ensure that the appropriate follow-up care and alternative treatments are provided. The sponsor and investigator will inform the FDA, IRB, IEC, DSMB, and any other applicable regulatory bodies as required by law.

11.12 Data Dissemination

Study data will be shared and registered with the National Institute of Health (NIH) and National Cancer Institute (NCI). All individual participant data will be anonymized or deidentified in compliance with HIPAA.

12 APPENDIX: SUPPORTING DETAILS

12.1 Clinical Laboratory Tests

Clinical laboratory tests will be performed to evaluate vital signs during screening to determine subject eligibility and at the time points specified in the schedule of activities (1.3) for study participants. Additionally, a 12-lead ECG will be performed at baseline and at the time points specified in the schedule for study participants to monitor for potential QTc prolongation or cardiac abnormalities (8.4.3). Antibodies will be assessed at the time points specified in the schedule to evaluate the immunogenicity of investigational treatments.

A Laboratory Manual will be provided at the site that specifies the procedures for collection, processing, storage, and shipment of samples. All samples will be analyzed by a licensed and Clinical Laboratory Improvement Amendments (CLIA)-certified laboratory. Abnormal laboratory test results should be repeated as soon as feasibly possible.

The clinical laboratory panels included in this study are hematology, serum chemistry, and urinalysis. The parameters included in each panel are summarized as follows:

Hematology:

White blood cell counts with differential
Red blood cell count
Hemoglobin
Hematocrit
Prothrombin time / International Normalized Ratio
Activated partial thromboplastin time
Platelet count
Mean corpuscular volume
Mean corpuscular hemoglobin concentration
Fibrinogen

Serum Chemistry: Tests for AST, ALT, ALP, direct bilirubin, indirect bilirubin, and total bilirubin must be conducted concurrently and assessed concurrently. Tests for FSH will only be performed to confirm the postmenopausal status of individuals who were clinically declared female at birth and are not surgically sterile.

Calcium
Chloride
Magnesium
Potassium
Sodium
Bicarbonate
Potassium

AST (aspartate aminotransferase)
ALT (alanine aminotransferase)
ALP (alkaline phosphatase)
Total bilirubin
Direct bilirubin
Indirect bilirubin
Thyroid stimulating hormone, free T4, free T3
Lipase
Gamma glutamyl transferase
Lactic dehydrogenase
Uric acid
Creatine
Blood urea nitrogen
Glucose
Albumin
Total protein
Triglycerides
Cholesterol
Amylase
FSH (follicle-stimulating hormone)

Urinalysis:

Color
Appearance
Specific gravity
pH
Protein
Glucose
Ketones
Blood
Bilirubin

Screening Tests: In addition to all the clinical laboratory tests described above, the following will be performed during the screening phase to determine subject eligibility:

Hepatitis B surface antigen, hepatitis B surface antibody, hepatitis B core antibody, IgM hepatitis B core antibody, hepatitis C antibody
HIV antibodies

Pregnancy Test: This is conducted during subject screening for biological females of childbearing potential (5.4.1).

Serum beta-human chorionic gonadotropin (β -hCG)

12.2 Country/Region-Specific Differences

Not applicable.

12.3 Prior Protocol Amendments

Not applicable. This protocol has not been amended.

13 APPENDIX: GLOSSARY OF TERMS AND ABBREVIATIONS

Abbreviation	Expansion
ADA	anti-drug antibodies
ADL	activities of daily living
AE	adverse event
CBC	complete blood count
cfDNA	cell-free deoxyribonucleic acid
CLIA	Clinical Laboratory Improvement Amendments
CR	complete response
CS	clinically significant
ctDNA	circulating tumor deoxyribonucleic acid
DLT	dose limiting toxicity
DMP	data management plan
dPCR	digital polymerase chain reaction
DSMB	data and safety monitoring board
ECOG	Eastern Cooperative Oncology Group
EDC	electronic data capture
ELISA	enzyme-linked immunosorbent assay
eCRF	electronic case report form
eTMF	electronic trail master file
FAAN	Food Allergy and Anaphylaxis Network
FDA	Food and Drug Administration
GCP	Good Clinical Practice
GDP	Good Documentation Practice
GDPR	General Data Protection Regulation
GEJ	gastroesophageal junction
GI	gastrointestinal
GLP	Good Laboratory Practice
HIPAA	Health Insurance Portability and Accountability Act
ICF	informed consent form
ICH	International Council for Harmonization
iCDP	immune confirmed disease progression
iCR	immune complete response
IEC	independent ethics committee
iPR	immune partial response
IRB	institutional review board
iSD	immune stable disease
iUDP	immune unconfirmed disease progression
iRECIST	immunotherapy response evaluation criteria in solid tumors

ISO	International Organization for Standardization
mAb	monoclonal antibody
MTD	maximum tolerated dose
NCS	not clinically significant
NGS	next-generation sequencing
NIAID	National Institute of Allergy and Infectious Diseases
ORR	overall response rate
PET/CT	positron emission tomography/computed tomography
PI	principal investigator
PK	pharmacokinetics
PR	partial response
RP2D	recommended phase II dose
SAE	serious adverse event
SAP	statistical analysis plan
SD	stable disease
SJS	Stevens-Johnson syndrome
SNV	single nucleotide variant
TEM	toxic epidermal necrolysis
TRAE	treatment-related adverse event
USP	United States Pharmacopeia
VAF	variant allele frequency
QMP	quality management plan

14 APPENDIX: REFERENCES

1. ctcae-v6.pdf. Accessed October 9, 2025. <https://dctd.cancer.gov/research/ctep-trials/for-sites/adverse-events/ctcae-v6.pdf>
2. Causes of Stomach Cancer - NCI. April 7, 2023. Accessed December 4, 2025. <https://www.cancer.gov/types/stomach/causes-risk-factors>
3. Chen J, Xu Z, Hu C, et al. Targeting CLDN18.2 in cancers of the gastrointestinal tract: New drugs and new indications. *Front Oncol*. 2023;13:1132319. doi:10.3389/fonc.2023.1132319
4. Baxter MA, Marinho J, Soto-Perez-de-Celis E, et al. Gastroesophageal adenocarcinoma in older adults: A comprehensive narrative review of management by the Young International Society of Geriatric Oncology. *J Geriatr Oncol*. 2022;13(1):7-19. doi:10.1016/j.jgo.2021.09.006
5. Balducci L. Systemic treatment of gastric and esophageal adenocarcinoma in elderly patients. *J Gastrointest Oncol*. 2015;6(1):75-78. doi:10.3978/j.issn.2078-6891.2014.093
6. Char SK, O'Connor CA, Ng K. Early-onset gastrointestinal cancers: comprehensive review and future directions. *Br J Surg*. 2025;112(7):znaf102. doi:10.1093/bjs/znaf102
7. Hu HM, Tsai HJ, Ku HY, et al. Survival outcomes of management in metastatic gastric adenocarcinoma patients. *Sci Rep*. 2021;11(1):23142. doi:10.1038/s41598-021-02391-z
8. PhD GB. Claudin-18.2 Emerges as a Novel Therapeutic Target in GI Malignancies | OncLive. October 14, 2025. Accessed October 17, 2025. <https://www.onclive.com/view/claudin-18-2-emerges-as-a-novel-therapeutic-target-in-gi-malignancies>
9. Nakayama I, Qi C, Chen Y, Nakamura Y, Shen L, Shitara K. Claudin 18.2 as a novel therapeutic target. *Nat Rev Clin Oncol*. 2024;21(5):354-369. doi:10.1038/s41571-024-00874-2
10. Mathias-Machado MC, de Jesus VHF, Jácome A, et al. Claudin 18.2 as a New Biomarker in Gastric Cancer—What Should We Know? *Cancers*. 2024;16(3):679. doi:10.3390/cancers16030679
11. Shitara K, Lordick F, Bang YJ, et al. Zolbetuximab plus mFOLFOX6 in patients with CLDN18.2-positive, HER2-negative, untreated, locally advanced unresectable or metastatic gastric or gastro-oesophageal junction adenocarcinoma (SPOTLIGHT): a multicentre, randomised, double-blind, phase 3 trial. *Lancet Lond Engl*. 2023;401(10389):1655-1668. doi:10.1016/S0140-6736(23)00620-7
12. Shah MA, Shitara K, Ajani JA, et al. Zolbetuximab plus CAPOX in CLDN18.2-positive gastric or gastroesophageal junction adenocarcinoma: the randomized, phase 3 GLOW trial. *Nat Med*. 2023;29(8):2133-2141. doi:10.1038/s41591-023-02465-7

13. Zhang M, Gong J, Wang J, et al. A phase I/II study of ASKB589 (anti-claudin 18.2 [CLDN18.2] monoclonal antibody) in patients with solid tumors. *J Clin Oncol*. 2023;41(4_suppl):397-397. doi:10.1200/JCO.2023.41.4_suppl.397
14. Sochacka-Ćwikła A, Mączyński M, Regiec A. FDA-Approved Drugs for Hematological Malignancies—The Last Decade Review. *Cancers*. 2021;14(1):87. doi:10.3390/cancers14010087
15. Baldo BA. Adverse events to monoclonal antibodies used for cancer therapy. *Oncoimmunology*. 2013;2(10):e26333. doi:10.4161/onci.26333
16. Baldo BA. Immune- and Non-Immune-Mediated Adverse Effects of Monoclonal Antibody Therapy: A Survey of 110 Approved Antibodies. *Antibodies*. 2022;11(1):17. doi:10.3390/antib11010017
17. Le Tourneau C, Lee JJ, Siu LL. Dose Escalation Methods in Phase I Cancer Clinical Trials. *JNCI J Natl Cancer Inst*. 2009;101(10):708-720. doi:10.1093/jnci/djp079
18. Kurzrock R, Lin CC, Wu TC, Hobbs BP, Pestana RC, Hong DS. Moving Beyond 3+3: The Future of Clinical Trial Design. *Am Soc Clin Oncol Educ Book*. 2021;(41):e133-e144. doi:10.1200/EDBK_319783
19. Sahin U, Türeci Ö, Manikhas G, et al. FAST: a randomised phase II study of zolbetuximab (IMAB362) plus EOX versus EOX alone for first-line treatment of advanced CLDN18.2-positive gastric and gastro-oesophageal adenocarcinoma. *Ann Oncol Off J Eur Soc Med Oncol*. 2021;32(5):609-619. doi:10.1016/j.annonc.2021.02.005
20. Sahin U, Schuler M, Richly H, et al. A phase I dose-escalation study of IMAB362 (Zolbetuximab) in patients with advanced gastric and gastro-oesophageal junction cancer. *Eur J Cancer*. 2018;100:17-26. doi:10.1016/j.ejca.2018.05.007
21. Hecht JR, Mitchell EP, Yoshino T, et al. 5-Fluorouracil, leucovorin, and oxaliplatin (mFOLFOX6) plus sunitinib or bevacizumab as first-line treatment for metastatic colorectal cancer: a randomized Phase IIb study. *Cancer Manag Res*. 2015;7:165-173. doi:10.2147/CMAR.S61408
22. Osumi H, Takahari D, Chin K, et al. Modified FOLFOX6 as a first-line treatment for patients with advanced gastric cancer with massive ascites or inadequate oral intake. *OncoTargets Ther*. 2018;11:8301-8307. doi:10.2147/OTT.S184665
23. Oken MM, Creech RH, Tormey DC, et al. Toxicity and response criteria of the Eastern Cooperative Oncology Group. *Am J Clin Oncol*. 1982;5(6):649-655.
24. Xie P, Zheng H, Chen H, et al. Tumor response as defined by iRECIST in gastrointestinal malignancies treated with PD-1 and PD-L1 inhibitors and correlation with survival. *BMC Cancer*. 2021;21:1246. doi:10.1186/s12885-021-08944-9

25. Borcoman E, Kanjanapan Y, Champiat S, et al. Novel patterns of response under immunotherapy. *Ann Oncol*. 2019;30(3):385-396. doi:10.1093/annonc/mdz003
26. Seymour L, Bogaerts J, Perrone A, et al. iRECIST: guidelines for response criteria for use in trials testing immunotherapeutics. *Lancet Oncol*. 2017;18(3):e143-e152. doi:10.1016/S1470-2045(17)30074-8
27. van Brummelen EMJ, Ros W, Wolbink G, Beijnen JH, Schellens JHM. Antidrug Antibody Formation in Oncology: Clinical Relevance and Challenges. *The Oncologist*. 2016;21(10):1260-1268. doi:10.1634/theoncologist.2016-0061
28. Manivannan V, Decker WW, Stead LG, Li JTC, Campbell RL. Visual representation of National Institute of Allergy and Infectious Disease and Food Allergy and Anaphylaxis Network criteria for anaphylaxis. *Int J Emerg Med*. 2009;2(1):3-5. doi:10.1007/s12245-009-0093-z